

Sino Biopharmaceutical (1177.HK): Dual Respiratory Milestones: AZ Out-licensing Deal and GSK Portfolio Expansion

Second BD deal with MNC; Licensing-out PDE3/4i to AZ: Sino Biopharm announced a licensing agreement with AstraZeneca for TQC3721 (a PDE3/4 inhibitor in phase 3), under which AZ obtains ex-China rights of TQC3721 for upfront payment of US\$200mn, up to US\$1.7bn in development, regulatory and commercial milestones, as well as tiered royalties ranging up to double-digit percentages based on net sales. In addition, AZ also gains global rights for certain future development programs for TQC3721, which are more innovative than ongoing clinical programs per company. TQC3721 has become the second China-originated PDE3/4 inhibitor to secure a major MNC partnership, following [Hengrui's deal with GSK](#) for its PDE3/4 inhibitor in 2025. The transaction marks Sino Biopharm's second major licensing-out deal with an MNC in 2026 following its earlier [Sanofi partnership](#), further demonstrating the company's growing ability to generate globally competitive assets from its internal R&D platform.

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Introducing two COPD drugs from GSK to strengthen respiratory portfolio: In addition to the TQC3721 out-licensing deal, the company also announced an expanding partnership with GSK following their [collaboration on bepirovirsen](#). Through the introduction of Trelegy (LABA/LAMA/ICS) and Anoro (LABA/LAMA) from GSK for an initiation period of 5.5 years, Sino Biopharm further enriches its respiratory portfolio, which now comprises 30+ marketed respiratory products supported by a dedicated 2,000+ respiratory salesforce covering over 9,000 hospitals nationwide. The two products generated combined global sales of nearly US\$5bn in 2025, while current China sales remain below Rmb1bn in total, suggesting significant room for penetration and market share gains. More importantly, the products fit well within Sino Biopharm's existing COPD treatment matrix and strengthen the company's ability to serve the full spectrum of COPD patients for acute onset and maintenance therapy. Looking ahead, we believe the commercial platform could create meaningful synergies with the company's internally developed respiratory pipeline, including TQC3721 (PDE3/4 inhibitor), TSLP monoclonal antibody (TQC2731) and IL-4R α monoclonal antibody (TQH2722).

Comparable efficacy of TQC3721 vs ensifentrine while differentiation from DPI formulation: Based on Phase 2 data, TQC3721 delivered placebo-adjusted improvements in FEV1 of approximately +100mL and +147mL at Week 4 for the 3mg and 6mg dose groups, respectively. This efficacy is largely comparable with the +124mL placebo-adjusted FEV1 improvement reported for first-mover ensifentrine 3mg in a non-head-to-head comparison (see [Exhibit 1](#)). Furthermore, TQC3721

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demonstrated an encouraging safety profile, with no treatment discontinuations reported in the study. Beyond efficacy, we believe the dry powder inhaler (DPI) of TQC3721 could be a key differentiator with better convenience. While ensifentrine is currently marketed as a nebulized therapy, Sino Biopharm is simultaneously advancing both nebulized and DPI formulations of TQC3721, with the DPI version currently in Phase 2 development with Phase 1 data readout to be expected in 2H26.

Evolving COPD treatment aiming for broader patient coverage / long-acting agents:

Compared with approved biologics for COPD such as dupilumab and mepolizumab, PDE3/4 inhibitors can potentially address a broader COPD population, as current biologics are largely focused on patients with type-2 inflammation and elevated eosinophil counts, which only account for a minority of COPD patients. However, the competitive landscape is evolving rapidly. AstraZeneca's tozorakimab (IL-33 mAb) has already delivered positive Phase 3 results and appears capable of treating a broad COPD population beyond traditional eosinophilic subgroups, potentially narrowing a key differentiation advantage of PDE3/4 inhibitors. Looking ahead, next-generation long-acting biologics may emerge as another disruptive force. For example, Keymed's CM512 (TSLP/IL13 bispecific antibody) is targeting a potential 3-6 month dosing interval and is expected to release Phase 2 COPD data by YE26, which could further shift the treatment paradigm toward broader coverage and improved convenience.

Estimates and valuation changes: Post deal announcement, we factor in the upfront payment of the AZ deal in 2H26, and also introduce sales estimates for Trelegy and Anoro, for which we expect to reach collective sales of Rmb1.2bn/1.4bn/1.7bn in 2026/27/28E (only reflecting 4Q sales for 2026). Therefore, our earnings estimates increased by +6.9%/+2.6%/+2.7% for 2026/27/28E. We also revise up PoLS for PDE3/4i from 80% to 100% given the global partner secured. Based on updated estimates and PoLS, our 12m-TP changed to HK\$8.41 from HK\$8.18.

Exhibit 1: Comparison of clinical data on COPD

	Ensifentrine			TQC3721		Dupilumab	Mepolizumab
MoA	PDE3/4			PDE3/4		IL-4Rα	IL-5
Trial no.	NCT04535986	NCT04542057	NCT03937479	NCT06527144	NCT03930732	NCT04456673	NCT04133909
Phase	Phase 3	Phase 3	Phase 2	Phase 2	Phase 3	Phase 3	Phase 3
Trial design							
Indication	COPD			CDPD	COPD with eosinophil ≥300/μL		COPD with eosinophil ≥300/μL
Intervention	Ensifentrine vs placebo			TQC3721 vs placebo	Dupilumab vs placebo		Mepolizumab vs placebo
Dosing	Ensifentrine 3mg BID			TQC3721 3/6mg BID	Dupilumab 300mg Q2W		Mepolizumab 100mg Q4W
Background therapy				LAMAs (28.8%), LABAs/LAMAs (71.2%)			
Sample size	760	789	416	241	939	935	804
Efficacy data							
annualized exacerbation rate	0.26 vs 0.41 (-36%)	0.24 vs 0.42 (-43%)			0.78 vs 1.10 (-30%)	0.86 vs 1.30 (-34%)	0.80 vs 1.01 (-21%)
change in FEV ₁ at week 4 (ml)			+124 (3mg)	+100ml (3mg), +147ml (6mg)			
change in FEV ₁ at week 12 (ml)	61 vs -26 (+87)	48 vs -46 (+94)			160 vs 77 (+83)	139 vs 57 (+82)	
SGRQ ¹ score	-6.2 vs -3.9	-4.5 vs -4.1		-5.09	-9.7 vs -6.4	comparable	
E-RS-COPD ² score	-2.2 vs -1.3	-2.1 vs -1.5			-2.7 vs -1.6		
Safety data							
Serious TEAE	6.7% vs 6.7%	5.6% vs 5.8%		1.23% (3mg) vs 0% (6mg) vs 1.25%	comparable for dupi vs pbo		comparable for mepo vs pbo
Any TEAE leading to death	0.4% vs 1.4%	0.8% vs 0.3%		0.0%	comparable for dupi vs pbo		comparable for mepo vs pbo
Discontinued treatment owing to TEAE	6.1% vs 6.4%	9.0% vs 10.0%		0.0%	comparable for dupi vs pbo		comparable for mepo vs pbo

¹St. George's Respiratory Questionnaire (SGRQ; range, 0 to 100, with lower scores indicating a better quality of life)

²Evaluating Respiratory Symptoms in COPD (E-RS-COPD; range, 0 to 40, with lower scores indicating less severe symptoms)

Source: American Journal of Respiratory and Critical Care Medicine, American College of Chest Physicians, NEJM

Price Target Risks and Methodology - Sino Biopharmaceutical

Our 12m SOTP-based TP is HK\$8.41. Our TP consists of 1) innovative pipeline: a DCF-based valuation of HK\$101.6bn; 2) generics: a valuation of HK\$50.2bn, based on

an exit P/E of 10.0x and a 5-year CAGR of 5%; 3) anlotinib and PD-(L)1: a DCF-based valuation of HK\$6.0bn. Key downside risks: 1) broader price cut on generics portfolio; 2) delay in regulatory approval of key products in the pipeline; 3) low return of R&D investment due to inappropriate resource allocation; 4) below-expectation ramp-up of innovative drugs.

Sino Biopharm is a leading pharma company in China that is engaged in both innovative and generic drugs. Its key focus areas include hepatitis, oncology, cardio-cerebral, orthopaedic, respiratory and analgesic indications. As the majority of the negative impact from VBP implementation has been reflected for hepatitis drugs since 2019, we expect a neutral impact from VBP going forward for the company as it has 100+ new generics expected to launch in the next three years to provide incremental sales. While we remain cautious on the company's R&D progress given the challenges of resource allocation within subsidiaries of the group, we see near-term drivers for the company from new to-be launched innovative drugs, including PD-1/L1, G-CSF, ROS1, ALKi, bepirovirsen and multiple biosimilars. We are Buy rated as we expect resilient earnings growth over the next two years and potentially more BD deals given its strong cash balance. The company is trading at the low end of its 5-year historical range. Key risks: 1) broader price cut on generics portfolio; 2) delay in regulatory approval of key products in the pipeline; 3) low return of R&D investment due to inappropriate resource allocation; 4) below-expectation ramp-up of innovative drugs.

1177.HK12m Price Target: **HK\$8.41**Price: **HK\$4.82**Upside: **74.5%****Buy****GS Forecast**

Market cap: HK\$90.4bn / \$11.5bn
Enterprise value:
HK\$105.7bn / \$13.5bn
3m ADTV: HK\$446.4mn / \$57.0mn
China
China Pharma & Biotech
M&A Rank: 3
Leases incl. in net debt & EV?: No

	12/25	12/26E	12/27E	12/28E
Revenue (Rmb mn) New	31,834.5	37,802.7	42,005.9	46,261.8
Revenue (Rmb mn) Old	31,834.5	36,768.2	40,571.3	44,540.4
EBITDA (Rmb mn)	7,466.5	11,125.3	11,972.5	13,396.2
EPS (Rmb) New	0.13	0.24	0.26	0.29
EPS (Rmb) Old	0.13	0.22	0.25	0.29
P/E (X)	39.3	17.7	16.1	14.3
P/B (X)	3.0	2.3	2.1	1.9
Dividend yield (%)	1.0	2.1	2.3	2.6
CROCI (%)	3.9	34.3	32.3	31.0
	6/25	12/25	6/26E	12/26E
EPS (Rmb)	0.19	(0.06)	0.12	0.11

Source: Company data, Goldman Sachs Research estimates, FactSet. Price as of 8 Jul 2026 close.

Disclosure Appendix

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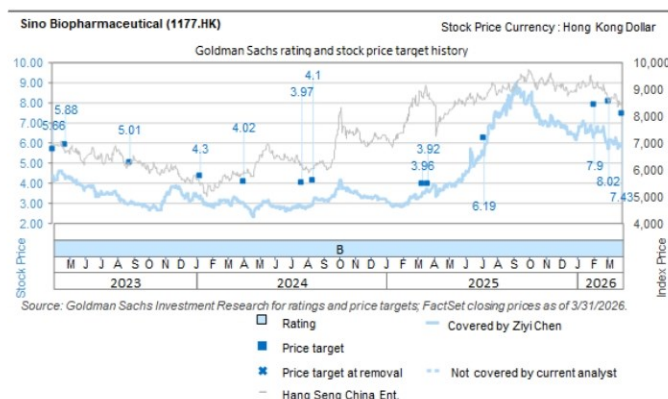
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Sino Biopharmaceutical (1177.HK)

Date of report	Target price (HK\$)	Closing price (HK\$)
02-Jun-26	8.18	4.88
11-May-26	8.05	5.66
30-Mar-26	7.43	5.85
04-Mar-26	8.02	5.71
05-Feb-26	7.90	6.43
08-Jul-25	6.19	5.37
23-Mar-25	3.92	3.60
11-Mar-25	3.96	3.56
13-Aug-24	4.10	2.95
23-Jul-24	3.97	2.83
16-Apr-24	4.02	2.54
03-Apr-24	4.02	2.92
09-Jan-24	4.30	3.32
27-Aug-23	5.01	3.01

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